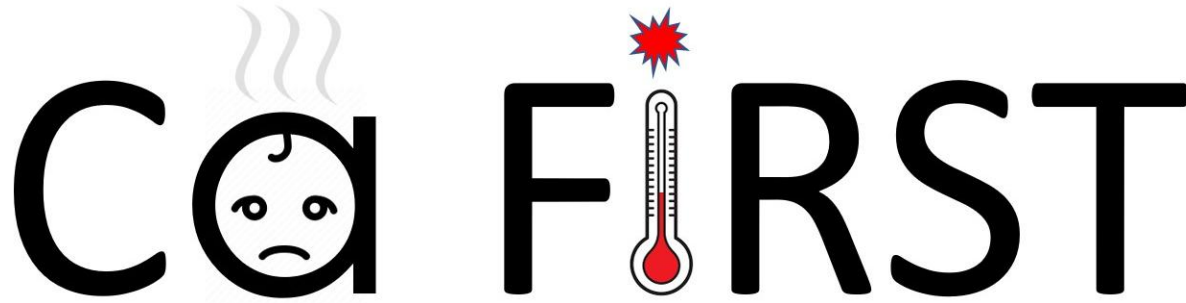




California Febrile Infant Risk Stratification Tool

Management of febrile infants 90 days old and younger
Modified for use in Kaiser Southern California

Abbreviated Flow Charts

Edited by Sean Fitzwater, MD MHS

Pediatric Infectious Diseases

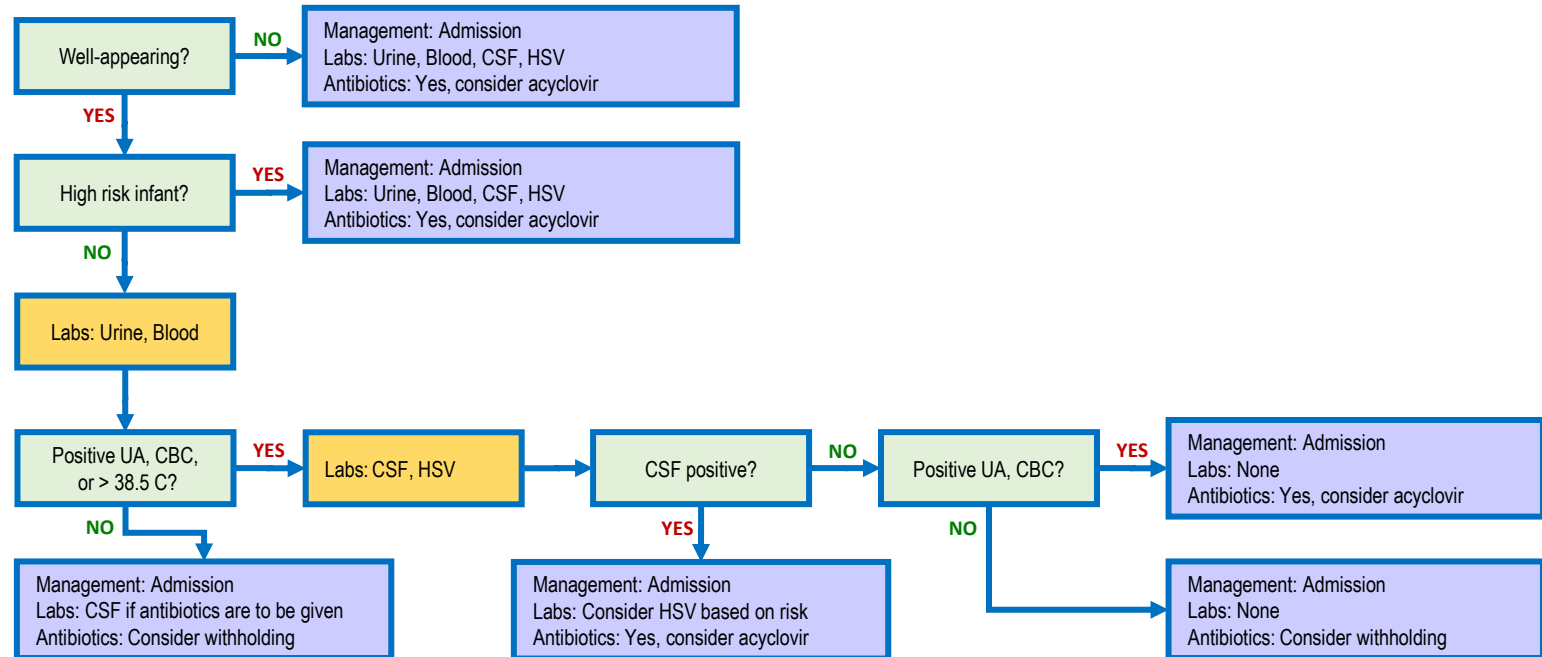
LAMC, PC, WH, KC

Updated 9/10/2025

Flowchart - Infants Aged 0 to 6 Days

[Link: Risk and Lab Details](#)

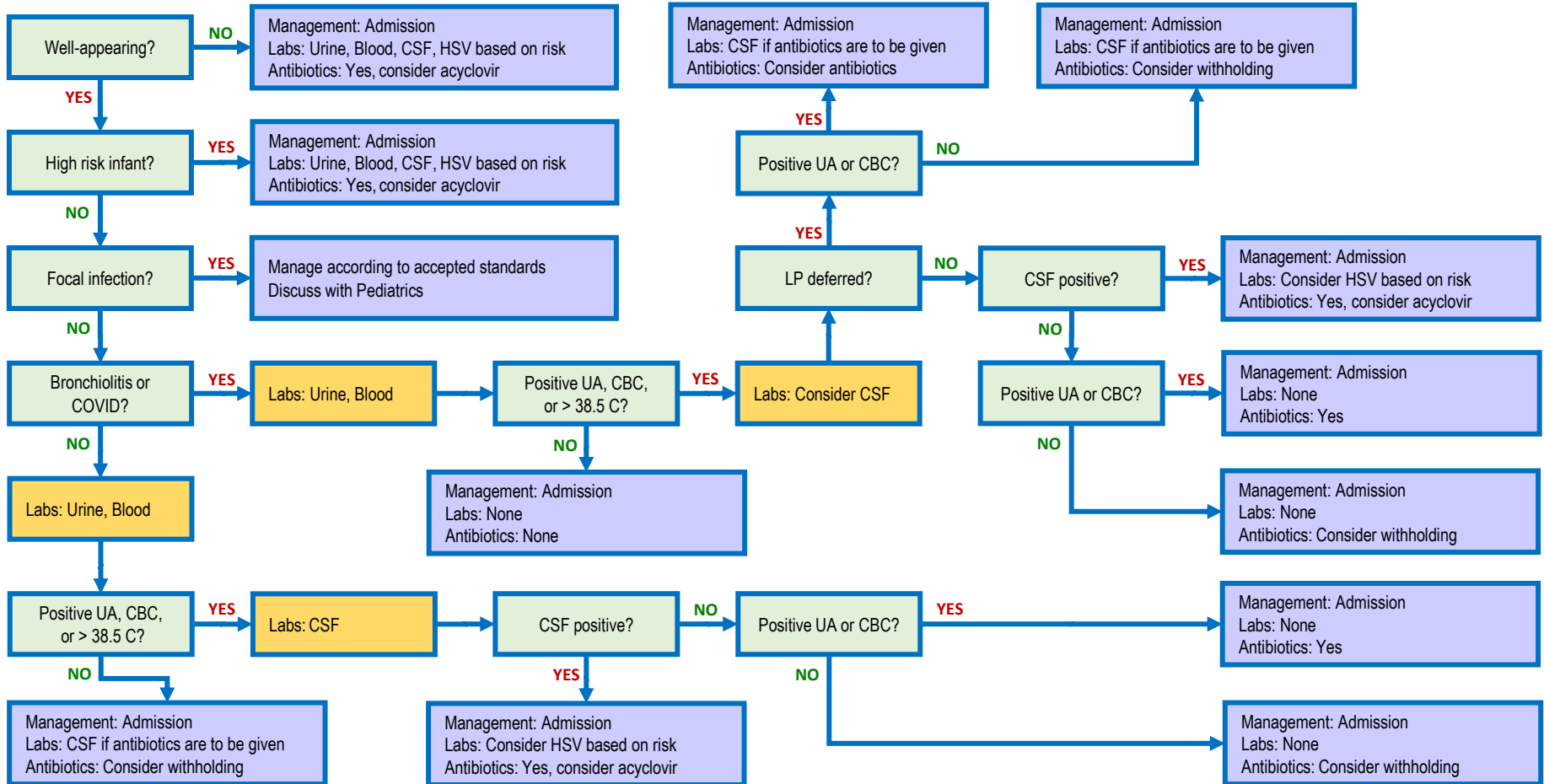
[Link: Antibiotic Details](#)



Flowchart - Infants Aged 7 to 21 Days

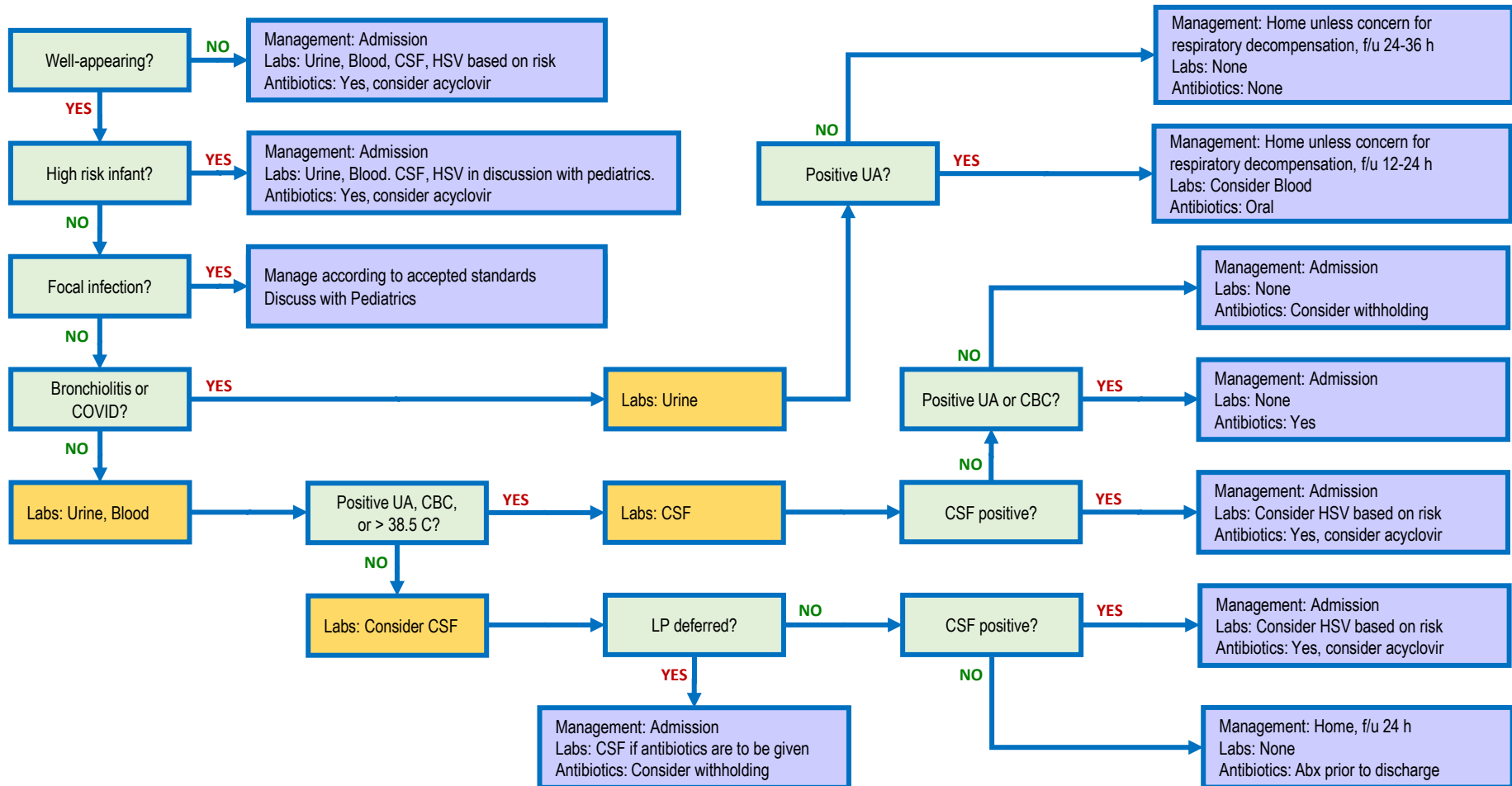
[Link: Risk and Lab Details](#)

[Link: Antibiotic Details](#)



Flowchart - Infants Aged 22 to 28 Days

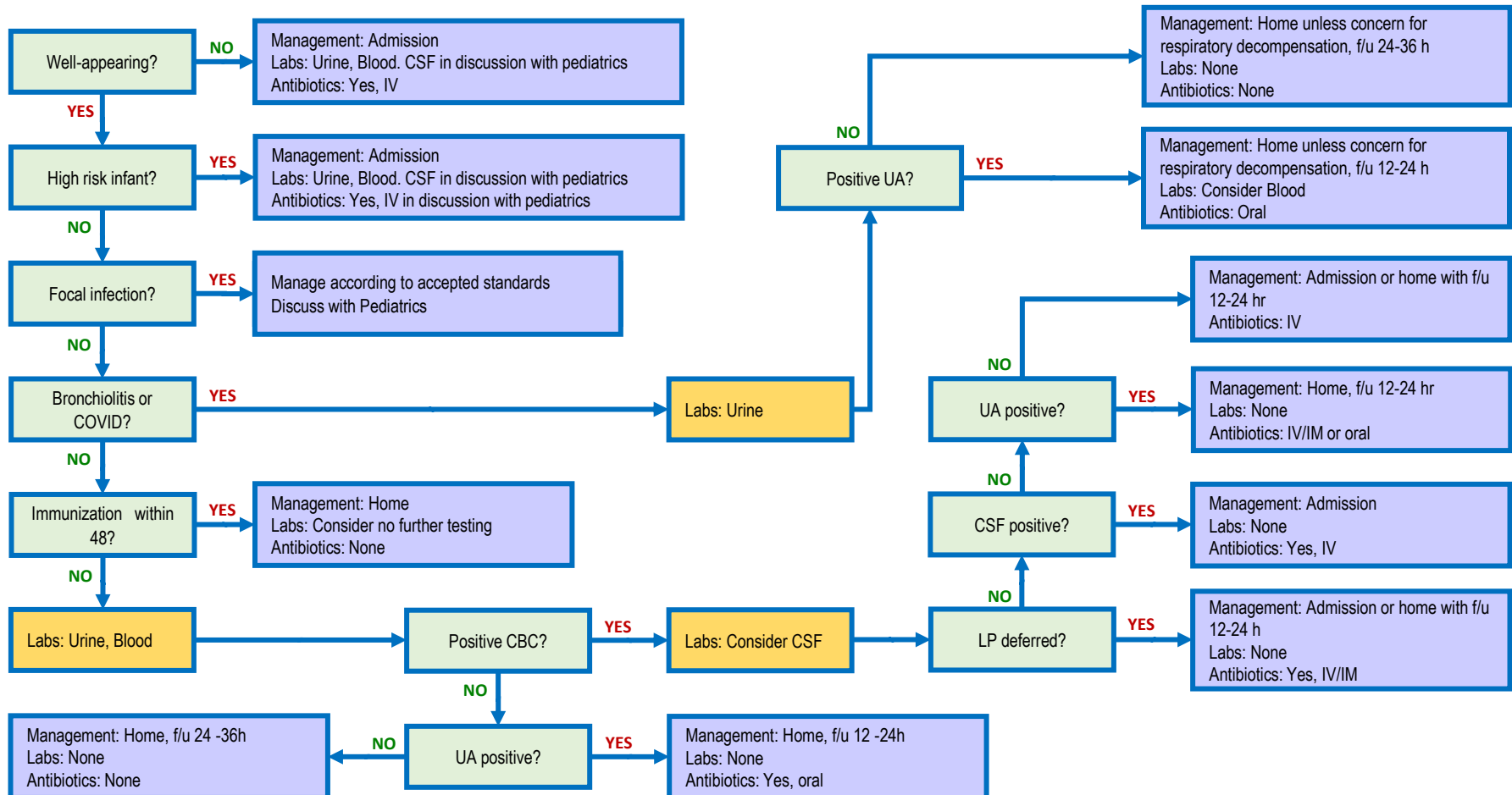
[Link: Risk and Lab Details](#)
[Link: Antibiotic Details](#)



Flowchart - Infants Aged 29 to 60 Days

[Link: Risk and Lab Details](#)

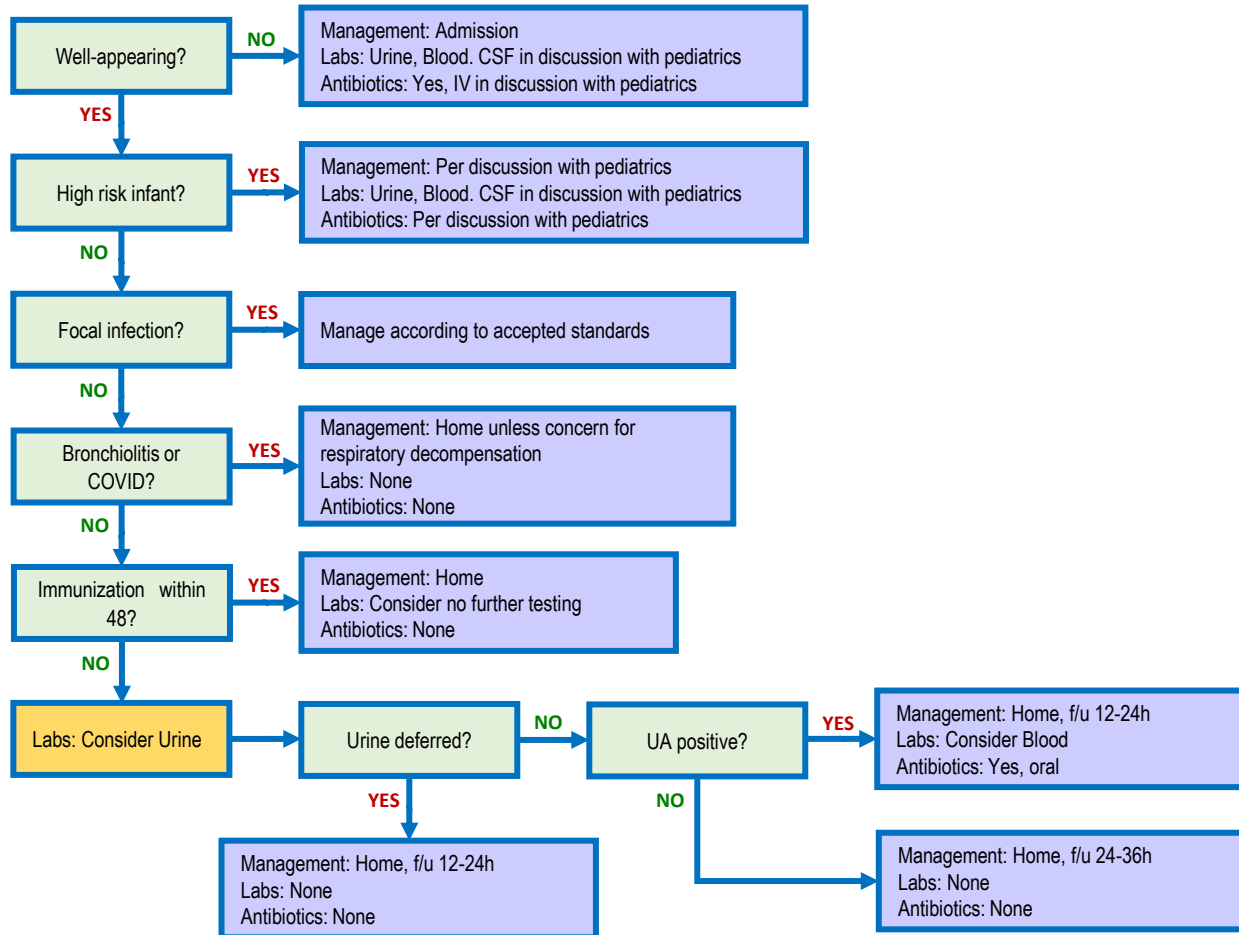
[Link: Antibiotic Details](#)



Flowchart - Infants Aged 61 to 90 Days

[Link: Risk and Lab Details](#)

[Link: Antibiotic Details](#)



Definitions and specifications of risks and laboratory work-up

Terminology

- “Consider” suggests that management is dependent on specific patient circumstances such as height of fever, duration fever, and the severity of other risk factors. A pediatrics consult may be helpful.

High risk infants

- Premature infants < 37 weeks gestational age. Well appearing infants 35-36 weeks gestational age at birth may not be high risk if no other risk factors are present.
- Younger than 14 days of age with a perinatal course complicated by maternal fever, infection, or antimicrobial use (excluding surgical prophylaxis or to treatment for GBS colonization).
- Neonatal course was complicated by surgery or infection.
- Documented or suspected immune compromise.
- Congenital or chromosomal abnormalities.
- Medically fragile infants requiring some form of technology or ongoing therapeutic intervention to sustain life.
- A high risk of herpes simplex infections based on history or exam (including vesicles, seizures, mucous membrane ulcers), or a maternal history of genital HSV.

Focal bacterial infections:

- Cellulitis, omphalitis, septic arthritis, osteomyelitis, etc.
- Symptoms consistent with viral upper respiratory infections are not included in this category.

Labs results concerning for a high risk of HSV

- CSF pleocytosis with a negative gram stain, leukopenia, thrombocytopenia, elevated ALT.

Lab specifications

- **Urine studies:** catheterized urinalysis AND catheterized urine culture
- **Blood studies:** complete blood count with differential AND bacterial culture. Consider ALT if unsure of HSV risk.
- **Cerebrospinal fluid studies:** cell count AND Gram stain AND glucose AND protein AND bacterial culture.
- **Herpes simplex virus studies:** CSF HSV PCR AND serum HSV PCR AND surface swab HSV PCRs (conjunctiva AND NP AND skin [including lanced vesicles] AND rectal) AND ALT.

Definitions of positive labs

- **Positive CSF:** White count with pleocytosis for age or traumatic tap.
- **Positive UA:** Positive leukocyte esterase on dipstick OR >5 WBC per high powered field (centrifuged) OR > 10 WBCs/mm³ (uncentrifuged).
- **Positive CBC:** Total white blood cell count < 5,000 OR total white blood cell count > 15,000 OR bands > 1,500.

Labs results concerning for a high risk of HSV

- CSF pleocytosis with a negative gram stain, leukopenia, thrombocytopenia, elevated ALT in infants <29 days old.
- Consider completing a full HSV work up and starting acyclovir in patients with elevated ALT

Antibiotics recommendations based on clinical suspicion and initial work up

0-6 days old

- **Urinary tract infection:** Ampicillin (50mg/kg q8h) PLUS [EITHER Ceftazidime (50mg/kg q12h) OR Gentamicin (4mg/kg q24h)]
- **No focus identified:** Ampicillin (50mg/kg q8h) PLUS [EITHER Ceftazidime (50mg/kg q12h) OR Gentamicin (4mg/kg q24h)]
- **Meningitis suspected:** Ampicillin (100mg/kg q8h) PLUS Ceftazidime (50mg/kg q12h), +/- Gentamicin (4mg/kg q24h)]
- **HSV suspected:** Acyclovir IV (20 mg/kg q8h)

7-21 days old

- **Urinary tract infection:** Ampicillin (50mg/kg q8h) PLUS [EITHER Ceftazidime (50mg/kg q8h) OR Gentamicin (4mg/kg q24h)]
- **No focus identified:** Ampicillin (50mg/kg q8h) PLUS [EITHER Ceftazidime (50mg/kg q8h) OR Gentamicin (4mg/kg q24h)]
- **Meningitis suspected:** Ampicillin (100mg/kg q8h) PLUS Ceftazidime (50mg/kg q8h), +/- Gentamicin (4mg/kg q24h)]
- **HSV suspected:** Acyclovir IV (20 mg/kg q8h)

22-28 days old

- **Urinary tract infection:** Ceftriaxone 50mg/kg q24h
- **No focus identified:** Ceftriaxone 50mg/kg q24h
- **Meningitis suspected:** Ampicillin (100mg/kg q8h) PLUS Ceftazidime (50mg/kg q8h), +/- Gentamicin (4mg/kg q24h)]
- **HSV suspected:** Acyclovir IV (20 mg/kg q8h)

29-90 days

- **Urinary tract infection:**
 - IV/IM: Ceftriaxone (50mg/kg q24h)
 - PO: Cefalexin (50-100 mg/kg divided TID)
- **No focus identified:** Ceftriaxone (50mg/kg q24h)
- **Meningitis suspected:** Ceftriaxone (50mg/kg q12h) PLUS vancomycin (60mg/kg/day divided q6h or q8h, interval based on chronological gestational age)